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Vitiligo

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O que é?

Vitiligo caracteriza-se pela diminuição ou falta de melanina (pigmento que dá cor à pele) em certas áreas do corpo, gerando manchas brancas nos locais afetados. As lesões, que podem ser isoladas ou espalhar-se pelo corpo, atingem principalmente os genitais, cotovelos, joelhos, face, extremidades dos membros inferiores e superiores (mãos e pés). O vitiligo incide em 1% a 2% da população mundial.

O que causa?

- Teoria neural

Vitiligo segmentar: incide geralmente sobre a região de um nevo (pinta) e é provocado por substâncias que destroem os melanócitos, células que produzem melanina.

- Teoria citóxica

A despigmentação da pele é provocada por substâncias como a hidroquinona presente em materiais como borracha e certos tecidos.

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O que causa?

- Teoria autoimune

Consiste na formação de anticorpos que atacam e destroem o melanócito ou inibem a produção de melanina. Parece estar associado a outras doenças auto-imunes, como diabetes e doenças da tireóide. Há ocorrência familiar em 20% a 30% dos casos.

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Review > Front Endocrinol (Lausanne). 2017 Oct 27:8:290. doi: 10.3389/fendo.2017.00290. eCollection 2017.

Vitiligo and Autoimmune Thyroid Disorders

Enke Baldini [1], Teresa Odorisio [2], Salvatore Sorrenti [1], Antonio Catania [1], Francesco Tartaglia [1], Giovanni Carbotta [1], Daniele Pironi [1], Roberta Rendina [3], Eleonora D'Armiento [3], Severino Persechino [4], Salvatore Ulisse [1]

Review > Br J Dermatol. 2012 Dec;167(6):1224-35. doi: 10.1111/j.1365-2133.2012.11198.x. Epub 2012 Oct 5.

The prevalence of thyroid disease in patients with vitiligo: a systematic review

C Vrijman [1], M W Kroon, J Limpens, M M G Leeflang, R M Luiten, J P W van der Veen, A Wolkerstorfer, P I Spuls

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Review > Oxid Med Cell Longev. 2022 Jan 19;2022:8796735. doi: 10.1155/2022/8796735. eCollection 2022.

The Promising Role of Chemokines in Vitiligo: From Oxidative Stress to the Autoimmune Response

Shan He [1], Jinhua Xu [1], Jinfeng Wu [1]

Affiliations + expand

PMID: 35096274 PMCID: PMC8791757 DOI: 10.1155/2022/8796735

Free PMC article

Abstract

Vitiligo is a common chronic autoimmune skin disorder featured with depigmented patches and underlying destruction of melanocytes in the lesional skin. Multiple factors and mechanisms have been proposed for the etiopathogenesis of vitiligo, among which oxidative stress has been widely accepted as a key factor in initiating melanocyte loss. The altered redox status caused by oxidative stress, including the overproduction of reactive oxygen species (ROS) and the decreased activity of the antioxidant system in the skin, surrenders the resistance of melanocytes to exogenous or endogenous stimuli and eventually impairs the normal defense mechanism, leading to the absence of melanocytes. Considering the important role of innate and adaptive immunity in vitiligo, there is mounting evidence revealing an association between oxidative stress and autoimmunity. Since the significant changes of chemokines have been documented in vitiligo in many recent studies, it has been suggested that ROS-mediated chemotactic signals are not only the biomarkers of disease progression and prognosis but also are involved in the pathogenesis of vitiligo by facilitating the innate and adaptive immune cells, especially melanocyte-specific T cells, trafficking to the lesional areas of vitiligo. In this review, we discuss the interaction between oxidative stress and autoimmune response orchestrated by chemokines, including CXCL16-CXCR6 axis, CXCL9/CXCL10-CXCR3 axis, and other altered chemokines in vitiligo, and we also try to provide insight into potential therapeutic options through targeting these pathways.

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Meta-Analysis > PLoS One. 2020 Nov 10;15(11):e0241445. doi: 10.1371/journal.pone.0241445. eCollection 2020.

Vitiligo and anxiety: A systematic review and meta-analysis

Assiya Kussainova [1], Laura Kassym [2], Almira Akhmetova [1], Natalya Glushkova [3], Ulugbek Sabirov [4], Saltanat Adilgozhina [5], Raikhan Tuleutayeva [6], Yuliya Semenova [7]

Affiliations

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An Bras Dermatol. 2009 Jan-Feb;84(1):41-5. doi: 10.1590/s0365-05962009000100006.

Vitiligo and emotions

[Article in English, Portuguese]

Lucas S C Nogueira [1], Pedro C Q Zancanaro, Roberto D Azambuja

Affiliations + expand

PMID: 19377758 DOI: 10.1590/s0365-05962009000100006

Abstract

On average, vitiligo affects one percent of the world population. More than 75% of the patients have negative self-image on account of the disease. The emotional impact of the dermatosis is frequently neglected by the caretaker, which has negative influence on therapy and prognosis. OBJECTIVE; To check the effect of vitiligo on patients emotions and discuss the mind-body interaction and its impact on the disease. METHODS; In their first medical visit, one hundred patients with various forms of vitiligo answered a question about which emotions were elicited by the presence of the spots. RESULTS; Eighty-eight percent of the patients with spots in exposed areas complained of unpleasant emotions versus twenty-seven percent of those with spots in unexposed areas. The most frequently referred emotions were fear, specifically of expansion of the spots (71%), shame (57%), insecurity (55%), sadness (55%) and inhibition (53%). CONCLUSION; Chronic illnesses generate in human beings a negative experience propitiated by the expectation of suffering. Besides appropriate scientific guidance, vitiligo patients need emotional comfort. Treatment outcomes and patients compliance to it, and even their resilience to face occasional therapeutic failures, rely on good physician-patient relationship. At a time when doctors make use of reputable therapeutic resources, it is indispensable that dermatologists become able to evaluate the patient in an integrative fashion.

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Tratamento

- Os tratamentos convencionais são longos e geralmente envolvem aplicações de pomadas à base de corticoides, loções e fototerapia (exposição ao sol com uso de substâncias fotossensibilizantes). Raramente ocorre cura definitiva das lesões, pois há áreas que apresentam maior dificuldade de recuperar a pigmentação. Quando o processo afeta mais de 50% do corpo a opção de tratamento pode ser a despigmentação total da pele.
- Segundo a Sociedade Brasileira de Dermatologia, os resultados do tratamento cubano que esteve em voga durante algum tempo não foram superiores aos da fototerapia convencional.
- É importante levar em conta o estado psicológico do paciente, visto que fatores emocionais podem agravar o aparecimento e evolução das lesões.

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Review > Dermatol Clin. 2017 Apr;35(2):257-265. doi: 10.1016/j.det.2016.11.014.

Vitiligo Pathogenesis and Emerging Treatments

Mehdi Rashighi [1], John E Harris [2]

Affiliations + expand

PMID: 28317534 PMCID: PMC5362109 DOI: 10.1016/j.det.2016.11.014

Free PMC article

Abstract

The pathogenesis of vitiligo involves interplay between intrinsic and extrinsic melanocyte defects, innate immune inflammation, and T-cell-mediated melanocyte destruction. The goal of treatment is to not only halt disease progression but also promote repigmentation through melanocyte regeneration, proliferation, and migration. Treatment strategies that address all aspects of disease pathogenesis and repigmentation are likely to have greatest efficacy, a strategy that may require combination therapies. Current treatments generally involve nontargeted suppression of autoimmunity, whereas emerging treatments are likely to use a more targeted approach based on in-depth understanding of disease pathogenesis, which may provide higher efficacy with a good safety profile.

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Desde o início da doença, o estresse oxidativo desempenha um papel significativo na promoção do aparecimento de vitiligo. Vários fatores levam à superprodução de espécies reativas de oxigênio (ROS) e, de forma colaborativa, causam o acúmulo de radicais livres nas células produtoras de pigmentos na pele (melanócitos).

A secreção de radicais livres é uma resposta ao estresse, gera inflamação e ativa excessivamente o sistema autoimune. Para a produção de melanina a célula precisa de grande quantidade de energia, produzida dentro da mitocôndria. Esta organela é considerada o indutor chave na produção dos radicais livres. Podem ser danificadas pelo contato com diversos estressores, como compostos químicos, alimentos alergênicos e falta de antioxidantes.

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PRIMEIRO PASSO

Protocolo Desinflama Conecta

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Review > Dermatol Clin. 2017 Apr;35(2):235-243. doi: 10.1016/j.det.2016.11.012.

The Role of Diet and Supplements in Vitiligo Management

Pearl E Grimes [1], Rama Nashawati [2]

Affiliations + expand

PMID: 28317532 DOI: 10.1016/j.det.2016.11.012

Abstract

Vitiligo is an autoimmune disorder that involves the interplay between oxidative stress and the immune system. Preliminary observations suggest that the presence of gluten in the diet may play a role in vitiligo development in some patients, but to date vitiligo-specific diets have not been studied. The role of oral supplements, including vitamins, minerals, and botanicals, is increasingly being investigated as adjuncts to conventional medical treatment due to their antioxidant and immunomodulatory activity. Studies suggest that many of these agents may have some efficacy as monotherapy, but more often as adjuncts to topical agents and phototherapy.

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Suplementos

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- Vit. C, Vit E, Vit. A
- Vit. D
- Glutationa
- Astaxantina
- Óleo de krill
- Betacaroteno
- Polypodium leucotomos
- Gingko biloba
- Ácido alfa lipóico
- Cúrcuma

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Review > Phytother Res. 2016 Aug;30(8):1243-64. doi: 10.1002/ptr.5640. Epub 2016 May 23.

Effects of Turmeric (Curcuma longa) on Skin Health: A Systematic Review of the Clinical Evidence

Alexandra R Vaughn [1], Amy Branum [2], Raja K Sivamani [3]

Affiliations + expand

PMID: 27213821 DOI: 10.1002/ptr.5640

Abstract

Turmeric (Curcuma longa), a commonly used spice throughout the world, has been shown to exhibit antiinflammatory, antimicrobial, antioxidant, and anti-neoplastic properties. Growing evidence shows that an active component of turmeric, curcumin, may be used medically to treat a variety of dermatologic diseases. This systematic review was conducted to examine the evidence for the use of both topical and ingested turmeric/curcumin to modulate skin health and function. The PubMed and Embase databases were systematically searched for clinical studies involving humans that examined the relationship between products containing turmeric, curcumin, and skin health. A total of 234 articles were uncovered, and a total of 18 studies met inclusion criteria. Nine studies evaluated the effects of ingestion, eight studies evaluated the effects of topical, and one study evaluated the effects of both ingested and topical application of turmeric/curcumin. Skin conditions examined include acne, alopecia, atopic dermatitis, facial photoaging, oral lichen planus, pruritus, psoriasis, radiodermatitis, and vitiligo. Ten studies noted statistically significant improvement in skin disease severity in the turmeric/curcumin treatment groups compared with control groups. Overall, there is early evidence that turmeric/curcumin products and supplements, both oral and topical, may provide therapeutic benefits for skin health. However, currently published studies are limited and further studies will be essential to better evaluate efficacy and the mechanisms involved. Copyright © 2016 John Wiley & Sons, Ltd.

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J Altern Complement Med. 2021 Apr;27(4):294-311. doi: 10.1089/acm.2020.0292.
Epub 2020 Dec 18.

A Systematic Review of Nutrition, Supplement, and Herbal-Based Adjunctive Therapies for Vitiligo

Anastasia Shakhbazova [1], Hera Wu [2 3], Cindy J Chambers [2 4 5], Raja K Sivamani [2 3 4 5 6]

Affiliations + expand

PMID: 33337930 DOI: 10.1089/acm.2020.0292

Abstract

Background: Vitiligo is an autoimmune skin condition that affects people globally anywhere, from <0.1% to more than 8% of individuals. The disease destroys skin melanocytes, resulting in a patchy depigmentation of the skin. About 50% of all patients develop the disease before their 20s. ***Methods:*** We systematically searched the literature and reviewed the evidence for the use of nutritional supplements and diet in the management of vitiligo. Embase and Medline were searched for diet, herbal, and nutrition-based clinical studies. Additional filters were applied that looked for controlled trial or randomized controlled trial and article or article in press or letter and English and clinical study. We selected clinical studies in humans that showed how diet or natural supplements can improve the symptoms of vitiligo in all of our searches. ***Results:*** There were 62 manuscripts that resulted from the PubMed search and 259 from the Embase search. A final of 26 studies were reviewed, and other supplemental case and case-control studies were used to introduce diet components that may influence either exacerbation or amelioration of vitiligo. Possible mechanisms of action are introduced for natural and supplemental interventions. ***Conclusion:*** Some of the supplements reviewed include *Gingko biloba*, oral *Polypodium leucotomos*, alpha lipoic acid, vitamins B12, D, and E, folic acid, phenylalanine, canthaxanthin, *Nigella sativa* oil, and other combined herbal bio-actives. Overall, the growing evidence is promising, but more studies are needed in this area to further explore the impact that supplements and diet can have on vitiligo management. The most promising therapies included oral phenylalanine as adjuvant therapy with UVA therapy, oral *G. biloba* as monotherapy, both of which can be used with other traditional therapies, and oral *P. leucotomos* with phototherapy or photochemotherapy.

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Review > Phytochemistry. 2022 Nov;203:113390. doi: 10.1016/j.phytochem.2022.113390.
Epub 2022 Aug 23.

Apigenin and its dermatological applications: A comprehensive review

Pantea Majma Sanaye [1], Mohammad Reza Mojaveri [2], Roohollah Ahmadian [3], Mehdi Sabet Jahromi [4], Roodabeh Bahramsoltani [5]

Affiliations + expand

PMID: 35998830 DOI: 10.1016/j.phytochem.2022.113390

Abstract

Apigenin is one of the abundant flavonoids in fruits and vegetables of human diet with several demonstrated health benefits. The aim of the present study is to provide an overview of the curren evidence regarding the effect of apigenin on different dermatological complications. Electronic databases including PubMed, Scopus, and Web of Science were searched to retrieve all papers assessing the dermatological effects of apigenin. Preclinical studies support beneficial effects of apigenin on UV-induced skin damage, vitiligo, dermatitis, wounds, skin aging, and some types of skin cancer. The compound mostly acts via inhibition of inflammation through suppression of pro-inflammatory cytokines and intracellular inflammatory mediators, as well as antioxidant properties such as improvement of endogenous antioxidant defense mechanisms. There are also some studies for the design and development of novel drug delivery systems for apigenin to improve its oral and topical bioavailability. Nevertheless, no clinical study has evaluated apigenin as a natural supplement for skin conditions. Considering the benefits of apigenin in preclinical models of dermatological disorders, as well as the acceptable safety of this compound, apigenin may be a future candidate to be used in dermatological disorders. Future clinical studies are needed to further confirm the safety and efficacy of apigenin in skin care products.

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RESEARCH LETTER | VOLUME 85, ISSUE 2, P506-508, AUGUST 2021

Antioxidant supplements in combination with phototherapy for vitiligo: A systematic review and metaanalysis of randomized controlled trials

Han Mi Jung, MD • Yu Seok Jung, MD • Ji Hae Lee, MD, PhD • Gyong Moon Kim, MD, PhD • Jung Min Bae, MD, PhD

Published: October 17, 2018 • DOI: https://doi.org/10.1016/j.jaad.2018.10.010 • Check for updates

Study	Country	Study design	Antioxidant component	Dosage	Frequency of phototherapy	Treatment duration (mo)	Enrolled patients	
							T	C
Study 1	Spain	DB RCT	Polypodium leucotomos	ND	PUVA, ND	3	10	9
Study 2	Italy	DB RCT	Lipoic acid (50 mg), vitamin C (50 mg), vitamin E (20 mg), polyunsaturated fatty acids (12%), and cysteine monohydrate (50 mg)	2 tablets/day	NBUVB, twice weekly	6	21	14
Study 3	Egypt	RCT	α-tocopherol	400 IU/day	NBUVB, thrice weekly	6	12	12
Study 4	Ecuador	DB RCT	Polypodium leucotomos	ND	NBUVB, twice weekly	6	7	6

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Review > BMC Dermatol. 2008 May 22;8:2. doi: 10.1186/1471-5945-8-2.

A systematic review of natural health product treatment for vitiligo

Orest Szczurko [1], Heather S Boon

Affiliations + expand

PMID: 18498646 PMCID: PMC2432048 DOI: 10.1186/1471-5945-8-2

Free PMC article

Abstract

Background: Vitiligo is a hypopigmentation disorder affecting 1 to 4% of the world population. Fifty percent of cases appear before the age of 20 years old, and the disfigurement results in psychiatric morbidity in 16 to 35% of those affected.

Methods: Our objective was to complete a comprehensive, systematic review of the published scientific literature to identify natural health products (NHP) such as vitamins, herbs and other supplements that may have efficacy in the treatment of vitiligo. We searched eight databases including MEDLINE and EMBASE for vitiligo, leucoderma, and various NHP terms. Prospective controlled clinical human trials were identified and assessed for quality.

Results: Fifteen clinical trials were identified, and organized into four categories based on the NHP used for treatment. 1) L-phenylalanine monotherapy was assessed in one trial, and as an adjuvant to phototherapy in three trials. All reported beneficial effects. 2) Three clinical trials utilized different traditional Chinese medicine products. Although each traditional Chinese medicine trial reported benefit in the active groups, the quality of the trials was poor. 3) Six trials investigated the use of plants in the treatment of vitiligo, four using plants as photosensitizing agents. The studies provide weak evidence that photosensitizing plants can be effective in conjunction with phototherapy, and moderate evidence that Ginkgo biloba monotherapy can be useful for vitiligo. 4) Two clinical trials investigated the use of vitamins in the therapy of vitiligo. One tested oral cobalamin with folic acid, and found no significant improvement over control. Another trial combined vitamin E with phototherapy and reported significantly better repigmentation over phototherapy only. It was not possible to pool the data from any studies for meta-analytic purposes due to the wide difference in outcome measures and poor quality ofreporting.

Conclusion: Reports investigating the efficacy of NHPs for vitiligo exist, but are of poor methodological quality and contain significant reporting flaws. L-phenylalanine used with phototherapy, and oral Ginkgo biloba as monotherapy show promise and warrant further investigation.

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Como usar ?

1. Escolher DOIS suplementos para VITILIGO + vitaminas
2. Pode acrescentar esses dois aos escolhidos de outras imersão
3. Usar na dose indicada do fabricante
4. Utilizar por até 3 meses e rotacionar
5. Pode tomar com chá verde, licorice, boldo, carqueja e gincko biloba

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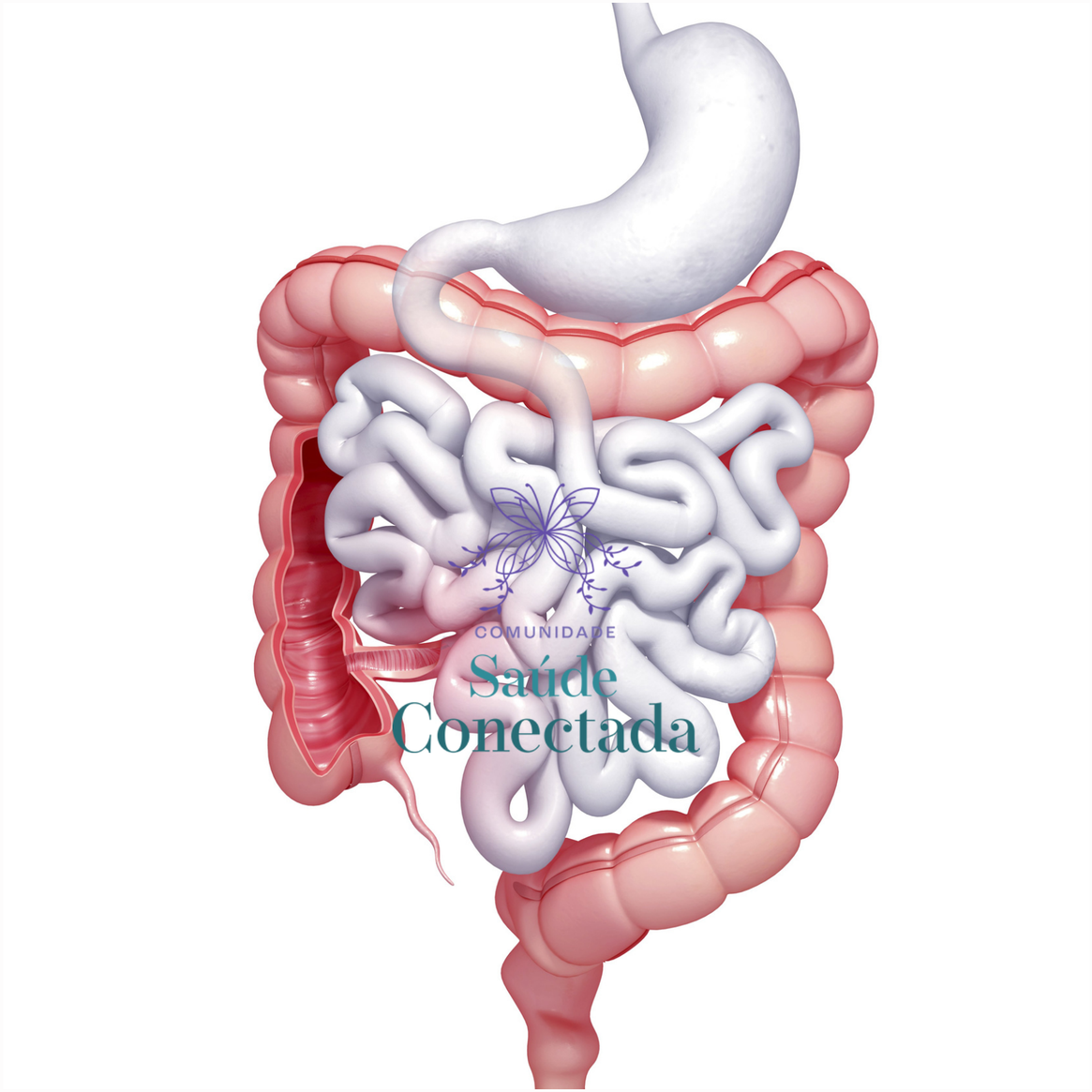

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Microorganisms. 2019 Nov; 7(11): 550.
Published online 2019 Nov 11. doi: 10.3390/microorganisms7110550
PMCID: PMC6920876
PMID: 31717915

The Skin and Gut Microbiome and Its Role in Common Dermatologic Conditions

Samantha R. Ellis,[1,2] Mimi Nguyen,[3] Alexandra R. Vaughn,[2] Manisha Notay,[2] Waqas A. Burney,[2,4] Simran Sandhu,[3] and Raja K. Sivamani[2,4,5,6,7,*]

Author information · Article notes · Copyright and License information Disclaimer

Abstract

Go to:

Microorganisms inhabit various areas of the body, including the gut and skin, and are important in maintaining homeostasis. Changes to the normal microflora due to genetic or environmental factors can contribute to the development of various disease states. In this review, we will discuss the relationship between the gut and skin microbiome and various dermatological diseases including acne, psoriasis, rosacea, and atopic dermatitis. In addition, we will discuss the impact of treatment on the microbiome and the role of probiotics.

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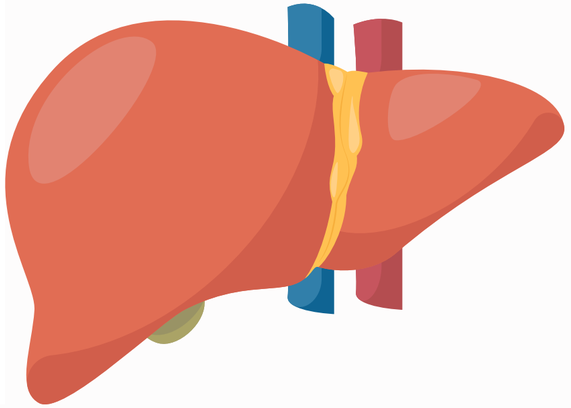
Review > J Eur Acad Dermatol Venereol. 2020 Dec;34(12):2766-2774. doi: 10.1111/jdv.16676.
Epub 2020 Jun 25.

Gastrointestinal involvement of primary skin diseases

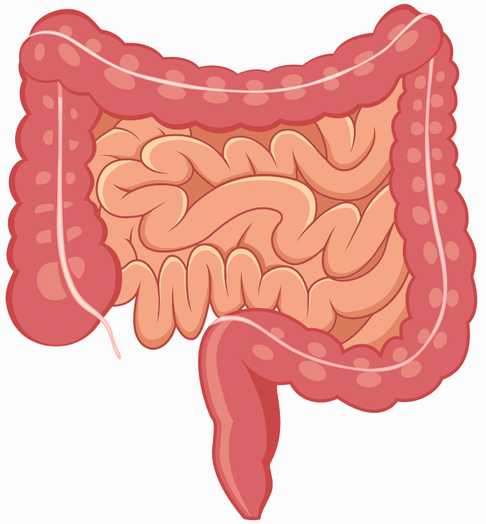
C-Y Lu [1 2], M-S Hsieh [3], K-C Wei [4], M Ezmerli [5], C-H Kuo [2 3], W Chen [6 7]



> Front Microbiol. 2020 Dec 14:11:592248. doi: 10.3389/fmicb.2020.592248. eCollection 2020.

Gut Microbial Dysbiosis and Plasma Metabolic Profile in Individuals With Vitiligo

Qingrong Ni [1], Zhubiao Ye [1], Yinghan Wang [1], Jianru Chen [1], Weigang Zhang [1], Cuiling Ma [1], Kai Li [1], Yu Liu [1], Ling Liu [1], Zheyi Han [2], Tianwen Gao [1], Zhe Jian [1], Shuli Li [1], Chunying Li [1]



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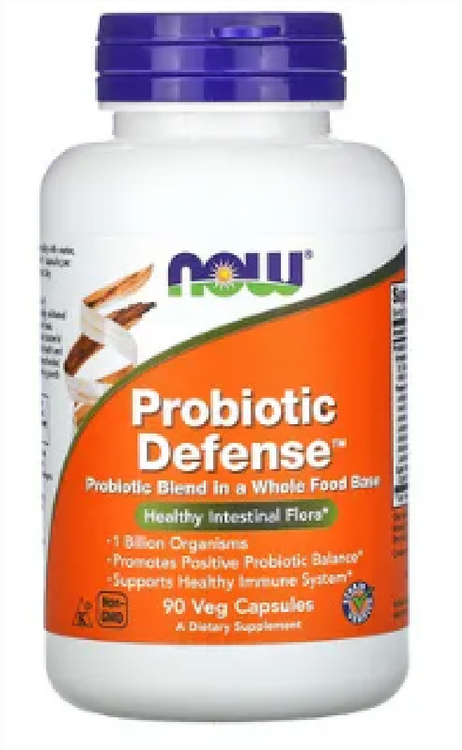
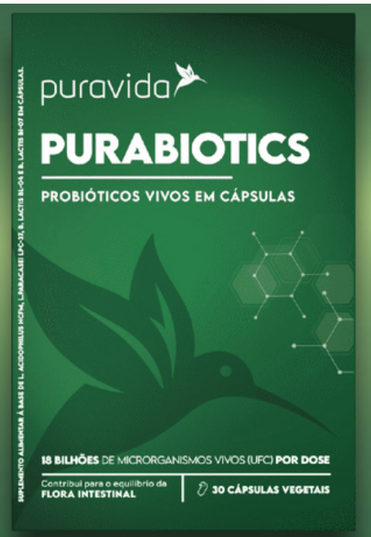


INGREDIENTES

Lactobacillus acidophilus NCFM (ATCC SD5221), Lactobacillus paracasei Lpc-37 (ATCC SD5275), Bifidobacterium animalis subsp lactis BI-04 (ATCC SD5219), Bifidobacterium animalis subsp lactis Bi-07 (ATCC SD5220), agente de massa celulose microcristalina, antiumectantes estearato de magnésio e dióxido de silício, cápsula (espessante hidroxipropilmetilcelulose).

NÃO CONTÉM GLÚTEN.



INFORMAÇÃO NUTRICIONAL

Porção de 1,9 g (3 cápsulas)

Quantidade por porção		%VD
Lactobacillus acidophilus NCFM	$4,5x10^9$ UFC	*
Lactobacillus paracasei Lpc-37	$4,5x10^9$ UFC	*
Bifidobacterium animalis subsp lactis BI-04	$4,5x10^9$ UFC	*
Bifidobacterium animalis subsp lactis Bi-07	$4,5x10^9$ UFC	*

Não contém quantidades significativas de valor energético, proteínas, carboidratos, gorduras totais, gorduras saturadas, gorduras trans, fibra alimentar e sódio. *VD não estabelecido.

SUGESTÃO DE USO

ADULTOS (MAIORES DE 19 ANOS), **INGERIR 3 CÁPSULAS AO DIA.**

USO NOTURNO, ANTES DE DORMIR

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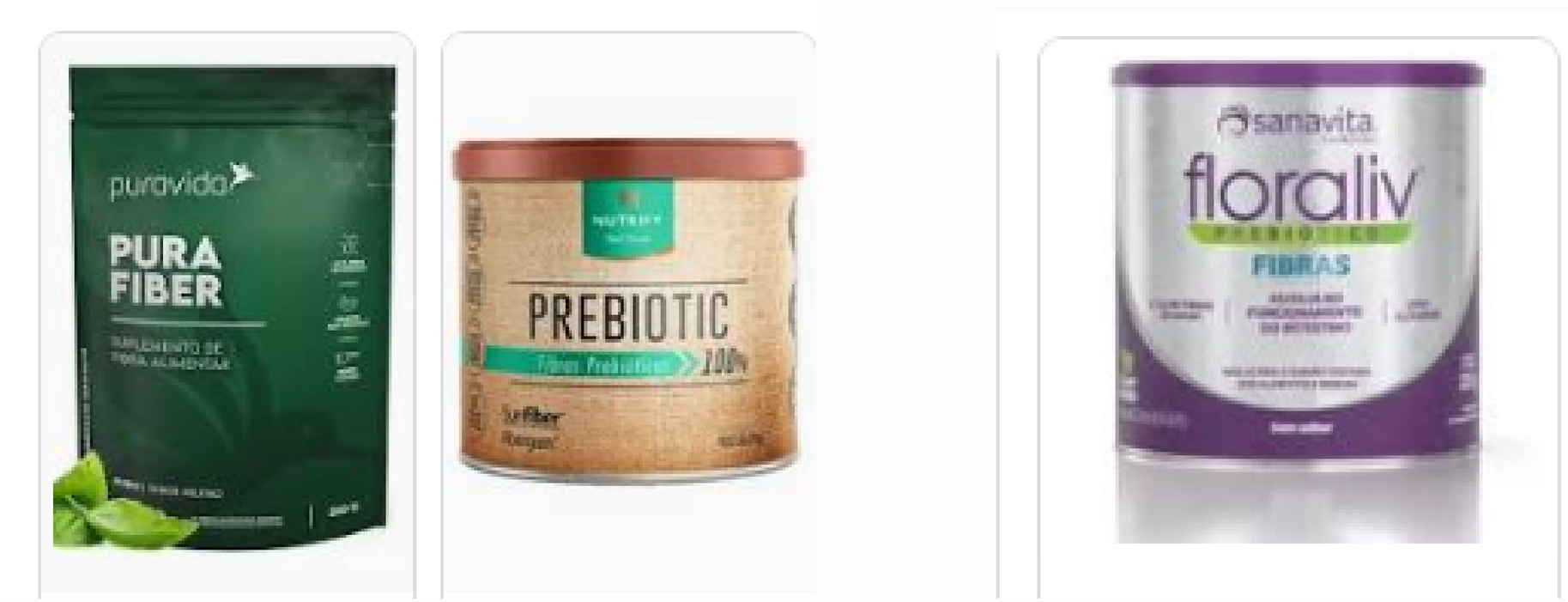

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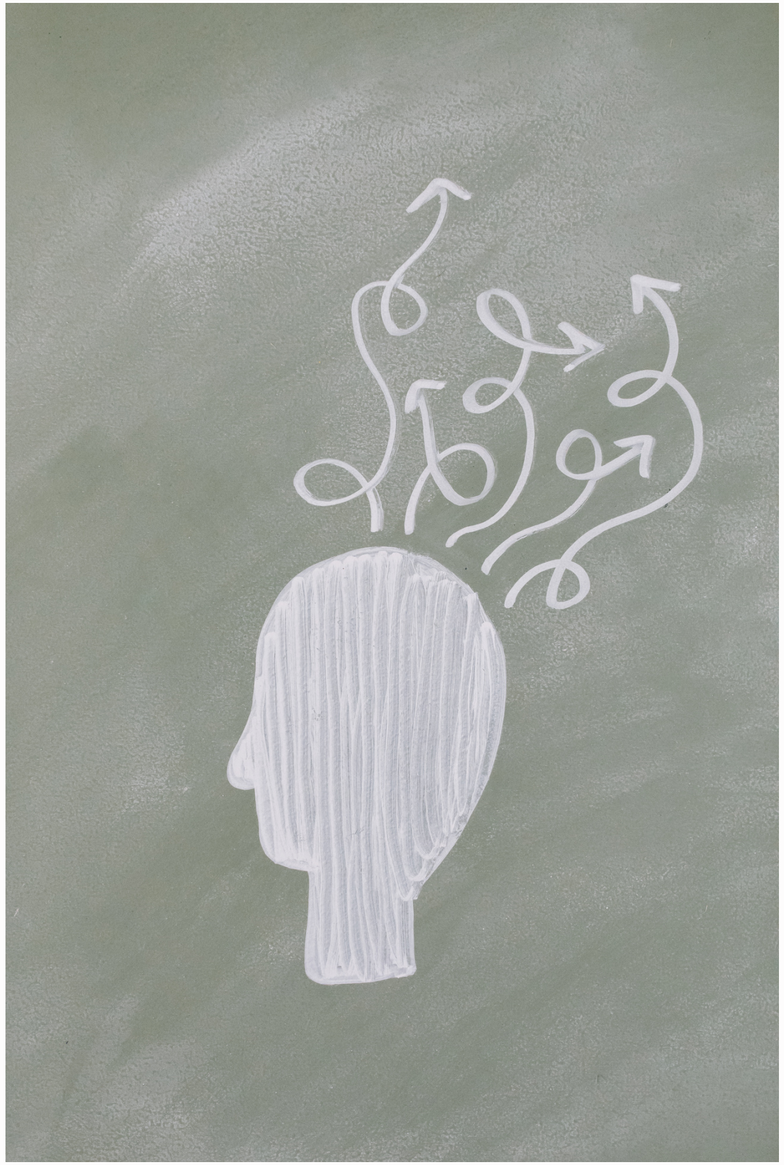

Review > Antioxidants (Basel). 2023 Jan 11;12(1):176. doi: 10.3390/antiox12010176.

Vitiligo and Mental Health: Natural Compounds' Usefulness

Luca Di Bartolomeo [1], Paolo Custurone [1], Natasha Irrera [2], Francesco Borgia [1], Federico Vaccaro [3], Francesco Squadrito [2], Mario Vaccaro [1]

Affiliations + expand

PMID: 36671038 PMCID: PMC9854903 DOI: 10.3390/antiox12010176

Free PMC article

Abstract

Vitiligo is an autoimmune dermatosis frequently associated with other comorbidities, such as mental health disorders. It is unclear if vitiligo triggers mental disorders or if mental disorders trigger vitiligo, but each one affects and worsen the other, if present at the same time. Both mental health disorders and vitiligo present a multifactorial pathogenesis and often require prolonged periods of therapy, sometimes with poor results. Given the possible link of common pathogenetic factors and the need of integrated therapies, the aim of this review is to look at natural compounds as possible supplements for both conditions. The results yielded show a possible role of these supplements in ameliorating both conditions, thus helping these patients to achieve a better quality of life and reduce the need for prolonged therapies. The limitations regarding the relative lack of in vivo studies, and the increasing need to lighten the burden of these chronic diseases, suggests that it is mandatory to proceed with further trials.

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CSC

Mascaras naturais

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Int J Dermatol. 2011 Mar;50(3):310-34. doi: 10.1111/j.1365-4632.2010.04744.x.

Integrating modern dermatology and Ayurveda in the treatment of vitiligo and lymphedema in India

Saravu R Narahari [1], Terence J Ryan, Kuthaje S Bose, Kodimoole S Prasanna, Guruprasad M Aggithaya

Abstract

Background: Globally, governments have recognized the growing popularity of Complementary and Alternative Medicines and the possibility of their combined use with biomedicine. Decisions within the Government of India have led to a conducive environment for conducting clinical studies, to achieve integration of more than one system of medicine, so that their combined benefits can be brought to bear on chronic, difficult-to-treat conditions.

Aim: To develop integrative dermatology treatment protocols for patients with long-standing skin diseases who have received treatment from many centers.

Materials and methods: A team of doctors from modern dermatology, Ayurveda, yoga therapy, and homeopathy studied recruited patients to develop mutual orientation on each therapeutic system and a working knowledge of approach to their clinical diagnosis. Six-hundred thirty-eight patients affected by lower limb lymphedema requiring skin care as a major part of treatment were treated integrating modern dermatology and Ayurveda. Three-hundred eighty-one vitiligo patients were examined and treated to understand the clinical presentations and treatment options in Ayurveda.

Results: A two-step cluster analysis performed by SPSS Version 16 showed average volume reductions of 13.3% and 23% on day 14, 19.7% and 31.1% on day 45, and 23.4% and 39.7% on day 90 of treatment in small and large lymphedematous limbs. Inflammatory episodes before the onset on this treatment was reported by 79.5% of our lymphedema patients, and 9.4% reported this at the end of three months after our treatment. Among vitiligo patients, we found that 39.6% of patients had kapha, 39.8% pitta, 10.8% had vatha and 0.52% has tridoshaja presentation. There are over 100 treatment options available in Ayurveda to treat vitiligo.

Discussion: Each system of medicine recognizes the same disease albeit with minor difference in description. Skin care procedures like washing and emollients restore the barrier function and skin health. We have converged Ayurvedic skin care with that of dermatology with an aim of achieving patient management that is better than that achievable by a single system alone. Overload of the lymphatic system due to loss of epidermal barrier function and consequent inflammation from bacteria and soil irritants is responsive to selected Ayurvedic herbal preparations.

Conclusion: It is evident that integration at the therapeutic level is possible, although the pathological basis is interpreted differently. Irrespective of background understanding of the given disease, a mutually oriented multisystem therapeutic team was able to effectively use medicines from more than one system of medicine and to develop guidelines for their prescription and a patient care algorithm.

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Clinical evaluation of the efficacy of *Shvitrahara kashaya* and *lepa* in vitiligo

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Para vitiligo pode ser usado o chá de cipó-de-São-João, sob a forma de banhos diários ou infusão alcoólica, aplicada sobre as manchas em dias alternados.

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Planta do cerrado é base de pesquisa para tratamento natural de vitiligo

Mama-cadela é fruto típico da região e conhecido popularmente pelas propriedades de repigmentação da pele

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Gengibre

O gingerol, uma substância rara presente no gengibre é responsável por trazer bons efeitos no tratamento do Vitiligo. Essa substância é capaz de melhorar o fluxo sanguíneo, permitindo então uma melhor performance, gradual, das células afetadas. Para que os efeitos positivos possam ser percebidos, é preciso manter uma rotina de uso dessa raiz.
Você pode fazer uso do gengibre em sucos e chás, por exemplo.
Aplique uma fatia recentemente cortada de raiz de gengibre na área afetada. Deixe até secar. Faça isso uma ou duas vezes por dia até ver uma melhoria. Você também pode beber suco de gengibre.

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A argila vermelha é muito utilizada em tratamentos estéticos e pode ser uma grande aliada para trazer uma nova pigmentação para a pele das pessoas que possuem vitiligo. A presença do cobre é a principal substância responsável pela restauração do pigmento. Além disso, a argila vermelha ajuda a melhorar o fluxo sanguíneo, o que pode promover o melhor funcionamento das células.
Em um recipiente, adicione a quantidade de argila vermelha que você julga necessário e acrescente o suco de gengibre para criar uma pasta. Dessa forma, você poderá aplicar a mistura na pele em uma rotina de tratamento.
Faça 1 x por semana

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Ingredientes necessários:

- Quantidade de argila vermelha que você julga necessário;
- Gengibre fresco.

Modo de preparo:
Lave bem o gengibre e rale-o com um ralador. Em seguida, utilize um espremedor para retirar o suco natural da raiz. Misture o suco natural do gengibre na argila vermelha e pronto! Agora é só fazer a aplicação na pele. Você pode estipular dias na semana em que vai realizar o tratamento e segui-lo verdadeiramente para que se possa obter bons resultados.

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Essa é uma planta familiar do gengibre e possui diversos benefícios específicos para a pele. A cúrcuma hidrata intensamente a pele e também é um ótimo pigmentador natural. Para o vitiligo você pode tomar um bom chá de cúrcuma com gengibre e limão. O limão, por ser rico em vitamina C, é um ótimo atuante para o antienvelhecimento da pele.

Ingredientes necessários:

- ½ quilo de cúrcuma crua;
- ¼ quilo de raiz de gengibre;
- Suco de 5 limões.

Modo de preparo:

Lave bem as raízes de cúrcuma e gengibre e as corte em pedacinhos pequenos. Retire o suco dos 5 limões. Em uma jarra, adicione todos os ingredientes e reserve na geladeira. Tome duas colheres dessa mistura antes de cada uma das suas refeições.

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A mistura de açafrão com óleo de mostarda também é muito eficaz no tratamento do Vitiligo . Misture cinco colheres de chá de açafrão com 250 ml de óleo de mostarda. Aplique esta mistura nas áreas descoloridas da pele duas vezes ao dia. Você precisa aplicá-lo todos os dias durante um ano para que o resultado seja mais eficiente.

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7. Neem:

As folhas de neem são excelentes para tratamento dos problemas de pele. Misture algumas folhas de neem com soro de leite para fazer uma pasta. Aplique nas manchas e deixe secar completamente. Lave com água morna. Faça isso todos os dias por algumas semanas.

Outra opção é aplicar óleo de neem diluído diariamente em uma colher de chá de outro óleo. Você também pode beber suco de neem ou comer 4 folhas de neem três vezes por dia antes das refeições.

Além de aplicar esses remédios, é recomendável que evite bagas, pois contêm hidroquinona, que funciona como agente despigmentante natural.

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Vinagre de cidra de maçã:

As propriedades antimicrobianas do vinagre de maçã podem destruir fungos que podem causar manchas brancas na pele. Misture quantidades iguais de vinagre de maçã e água. Use esta mistura para lavar a pele afetada duas vezes ao dia.

Faça isso todos os dias durante mês e observe.
Você também pode beber um copo de água misturado com uma colher de vinagre de maçã antes da refeição.

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Sementes de rabanete:

Um remédio ayurvédico é aplicar uma pasta de sementes de rabanete com vinagre. Esmague 25 gramas de sementes de rabanete e misture com 2 colheres de chá de vinagre de maçã para fazer uma pasta. Aplique a pasta na área afetada por 30 minutos e enxague com água morna. Faça isso todos os dias até ver resultados.

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Óleo de coco:

Suaviza a pele e cura a inflamação crônica, isso ajuda no tratamento do Vitiligo pois promove a repigmentação da pele. Além disso, tem propriedades antifúngicas e antibacterianas.

Basta aplicar o óleo de coco extra virgem nas manchas brancas duas ou três vezes por dia durante 4 semanas e depois rotacionar com outra técnica que ensino aqui

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Graças às suas propriedades imunomoduladoras e antioxidantes, o ginkgo biloba pode reduzir a progressão da doença.

A dose geral é de 40 a 80 mg de ginkgo biloba três vezes ao dia.

Mas sugiro que use o chá / infusão.

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Exemplo de suplementação

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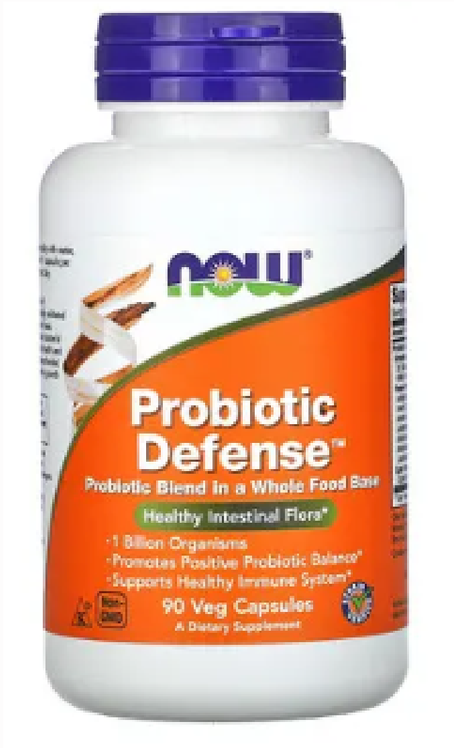

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